



SPECIMEN

- 1 Bladder, uterus, ovaries and cervix.
- 2 Left ureteric resection margin
- 3 Right ureteric resection margin

CLINICAL DETAILS

Cystectomy for bladder cancer.

MACROSCOPY

- 1 - Radical cystectomy specimen with attached uterus, ovaries and anterior vaginal wall.

The bladder is approximately 90 mm diameter and has associated adipose tissue around it. On the attached peritoneal surface there is an irregular haemorrhagic mass approximately 15 mm diameter, suggesting extension of tumour. Opening the bladder reveals a fungating mass arising from the posterior wall and with extensive surface necrosis. The tumour is approximately 70 mm diameter and is attached by a broad base. In addition, the background mucosa has a 20 mm separate mass and also two further 4 mm flattish nodules. Section through the mass shows extension through the bladder wall to come close to the peritoneal surface. Overall tumour thickness is approximately 40 mm. Away from the main and smaller tumour nodules the bladder mucosa appears normal, though there is marked trabeculation of the bladder. The urethra measures 25 mm and has normal surface mucosa, but there is 5 mm nodule between the midpoint of the urethra and the anterior vaginal wall.

The uterus measures 60 mm supero-inferiorly, and there is no apparent abnormality of cervix, endometrium or myometrium. The left ovary measures 20 mm, the right 18 mm, and both show senile atrophy with no further abnormality of them or of the fallopian tubes.

- 2 - This is 5 mm of ureter.

- 3 - This is 9 mm of ureter.

Block designation:

Specimen 1

A = tumour peritoneum J = background mucosa anterior

B = haemorrhagic area on peritoneum K = background mucosa

C = tumour and adjacent normal L = urethra midpoint

D-E = composite full depth of tumour M = urethra margin

F = tumour & background mucosa N = cervix

G = tumour left lateral with invasion of fat O = endometrium

H = 20 mm separate nodule tumour P = left ovary and tube

I = 5 mm nodules of tumour Q = right ovary and tube

MICROSCOPY

- 1 - The tumour is composed of extensive sheets of undifferentiated carcinoma cells with small areas of necrosis. In addition, some areas show squamous differentiation with keratinisation. These features would be consistent with transitional cell carcinoma, grade 3, with squamous differentiation. Similar tumour is seen in the main mass and in the separate nodules.

The tumour invades right through the bladder wall to reach adipose tissue, and there is also extravesical lymphovascular invasion. The haemorrhagic tissue on the peritoneal surface is further tumour. In general the tumour seems 2 or 3 mm clear of the peritoneal surface, but this peritoneal deposit of tumour suggests there may be a focus with invasion, or local lymphatic spread to the surface.

CHF ICD-O-3
Carcinoma, urothelial NOS 8/20/13
path Carcinoma, transitional cell NOS
& squamous cell (8670/3) Code to highest 8/20/13
Site: Bladder, posterior wall
C67.4
OK 5/2/14

PW T3S, 10% squamous.

In addition, there is a nodule of tumour in the septum between midpoint urethra and anterior vaginal wall.

Two tiny lymph nodes in perivesical adipose tissue show reactive changes without tumour. The usual regional lymph nodes have not been supplied.

Away from the tumour, the bladder epithelium is either normal, or shows a minor degree of glandular differentiation. The urethra mucosa and resection margin is normal.

The uterus shows no histological abnormality apart from atrophy, and the same applies to the ovaries.

2 & 3 - These samples show normal ureters.

SUMMARY

Radical cystectomy with hysterectomy; transitional cell carcinoma of bladder, WHO 1973 Grade 3, pT3b, pNx with positive peritoneal surface.

Pathologists -

Criteria	Yes	No
Diagnosis Discrepancy		/
Primary Tumor Site Discrepancy		/
HIPAA Discrepancy		/
Prior Malignancy History		/
Dual/Synchronous Primary Noted		/
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	SC	Date Reviewed: 1/10/14